Horizon: A Global Collaborative Forum on Cancer Care

- **Edition Focus:** *Cholangiocarcinoma – Bridging Borders in Biliary Cancer Management*
- **Organized by:** Jarurat Care Foundation
- **Date:** June 21, 2025
- **Format:** Online (Zoom)
- **Attendees:** Medical oncologists, surgical oncologists, GI specialists, researchers, caregivers, and healthcare workers from India and abroad
- **Supported by:** AstraZeneca · Servier · Datar Cancer Genetics

---

🌟 Purpose and Vision

This was the inaugural session of **Horizon**, the flagship global cancer series by *Jarurat Care Foundation (JCF)*, aimed at addressing neglected cancers through collaboration, access, and innovation.

Founder Priyanka Joshi opened the event by sharing JCF's founding story rooted in personal experience. She emphasized the pain of navigating biliary tract cancer (BTC) with little guidance, and how that experience inspired a robust, scalable support system for other families across India.

JCF's core initiatives include:

- **Multilingual AI-based support tools:** chatbot for treatment navigation, referrals, emotional support
- **Patient registry and research networks** focused on gallbladder and bile duct cancers
- **Pilot initiatives:** early screening, regional outreach, and myth-busting
- **Collaborative trial participation:** including access to biomarker testing and precision therapy trials
- Building a **Pan-India care navigation ecosystem** for underserved cancer groups

---

👩‍💼 Leadership & Faculty

🔷 International Keynote Speaker

- **Dr. Rachna Shroff**, Chief of GI Medical Oncology and Associate Director, University of Arizona Cancer Center (USA): A global authority on biliary tract cancer (BTC), clinical trialist for SWOG 1815 and KEYNOTE-966.

🔷 Co-Chairs

- **Dr. Milind Javle**, Clinical Medical Director, GI Medical Oncology, MD Anderson Cancer Center (USA)

- **Dr. Sewanti Limaye**, Director of Medical & Precision Oncology, Sir H. N. Reliance Foundation Hospital (India)

Their welcome highlighted how global-local collaboration is key to accelerating treatment access and awareness in BTC — especially in countries like India where cholangiocarcinoma and gallbladder cancers have distinct epidemiology and socioeconomic barriers.

🎙️ Keynote Talk by Dr. Rachna Shroff

Title: *International Advances in Cholangiocarcinoma: What's Changing and Why It Matters Globally*

Dr. Shroff provided a sweeping and data-rich overview of both standard treatments and the emerging targeted therapy revolution. Key themes included:

🔹 1. Standard of Care: First-Line

- The **TOPAZ-1 trial** has redefined front-line therapy for BTC:
  - *Gemcitabine + Cisplatin + Durvalumab* significantly improved OS.
  - 3-year OS: 14.6% (vs. 6.9% in chemo-only arm)
  - Benefits observed regardless of PD-L1, MSI, or biomarker profile.
- The **KEYNOTE-966 study** further validated immunotherapy with pembrolizumab.

🔹 2. Second-Line Therapies: Modest Gains

- The **ABC-06 study** supports *FOLFOX* post-progression.
- **Liposomal irinotecan** shows limited added benefit.
- Many patients in India lose out due to access delays and lack of navigation during progression.

🧬 Precision Medicine in BTC: The Molecular Era

Dr. Shroff emphasized that **BTC is no longer a single disease**. It's a cluster of **molecularly unique subtypes**, and **precision oncology** is now the future.

🔹 IDH1 Mutation (~10–15% of intrahepatic CCA)

- **Ivosidenib (ClarIDHy Trial)**: improved PFS, well tolerated.
- Now being studied in combination with GemCis and Durvalumab.
- Vital to test early—waiting for progression can close treatment windows.

🔹 FGFR2 Fusions (also 10–15%)

- Agents like:
  - **Pemigatinib**
  - **Futibatinib**
  - **Tinagotinib** (newest; activity even after FGFR resistance)

🔹 **HER2 Overexpression**

- HER2 amplification is common in gallbladder cancer.
- **Zanidatamab** – 41.3% ORR in BTC (HORIZON-BTC-01)
- **Trastuzumab Deruxtecan (T-DXd)** – now tumor-agnostic approval for HER2 IHC 3+
- Early HER2 testing increasingly essential

🔹 **KRAS G12C / BRAF V600E**

- Tumor-agnostic approvals now include BTC:
  - **Sotorasib** for KRAS G12C
  - **Dabrafenib + Trametinib** for BRAF V600E

🔹 **Other emerging targets:**

- **NRG1**, **RET**, **MTAP loss**, **HER3**, **Claudin 18.2**, **TROP2**
- ADCs and bispecific antibodies are entering BTC trials

Dr. Shroff urged for **molecular profiling at diagnosis** — not after multiple progressions — and **India's integration into global basket trials** like **ARBOR-BTC (US)** and **SAFIR-ABC (UK)**.

🧪 India-Specific Challenges & Trials

Dr. Shroff reviewed the **ACCELERATE trial** — one of the rare Indian trials in BTC.

- Compared chemo + chemo radiation vs chemo alone in gallbladder cancer post-surgery
- Result: No survival benefit from radiation
- Major bottlenecks: **slow accrual**, **logistics**, **lack of funding**, **low genomic profiling rates**

She emphasized the **urgent need for Indian real-world data**, **genomic registries**, and **infrastructure support** for trials, especially for gallbladder cancer which is more common in the Indian subcontinent.

🧠 CASE PRESENTATIONS & EXPERT PANEL REFLECTIONS

🟨 Case 1: Advanced Gallbladder Cancer (GBC)

- **Age/Gender:** 39-year-old woman
- **Initial Presentation:** Obstructive jaundice, significant weight loss, right upper quadrant discomfort
- **Imaging:** Multiple liver metastases, gallbladder wall thickening, regional lymphadenopathy
- **Histopathology:** Moderately differentiated adenocarcinoma consistent with GBC
- **Staging:** Metastatic GBC (Stage IV)

Treatment Journey:

- **First-Line:** Gemcitabine + Cisplatin + Durvalumab (per TOPAZ-1 protocol)
- Good tolerance and partial radiological response initially
- **Progression:** Noted after 8 cycles — new hepatic lesions and CA19-9 rise
- **Second-Line:** Switched to FOLFOX regimen (per ABC-06 trial guidance)

Discussion Points:

- **Biliary Drainage Approach**
  - **PTBD vs ERCP**:
    - Experts agreed ERCP is preferable due to lower infection risk and better patient comfort.
    - In complex hilar involvement, PTBD may be used selectively.
    - Multidisciplinary biliary decompression strategies were emphasized.
- **Role of Surgery in Advanced Disease**
  - Panel agreed resectability was not an option at presentation, but **conversion surgery** is possible in rare cases post durable response.
  - However, **timing and patient selection are critical**. Surgery should only be considered after reassessment post-response and in MDT review.
- **Biomarker Testing**
  - Dr. Prashant Kumar highlighted:
    - HER2 amplification is often missed but highly relevant in Indian GBC.
    - MSI-H, PD-L1, and NTRK testing can open avenues for immunotherapy or tumor-agnostic therapy.
    - **Genomic profiling should be upfront**, especially in younger patients.
- **Local Consolidative Therapy**
  - Dr. Prasad Raj Dandekar discussed:
    - Stereotactic body radiation **(SBRT)** can be used for oligo progression to delay systemic switching.
    - Indicated only in well-selected, low-burden disease with controlled systemic symptoms.
    - Coordination with medical oncology is essential

- **Clinical Trials & Access Gaps**
  - Indian patients often **progress before trials open**, or cannot afford second-line options like targeted therapies.
  - Horizon was urged to push for **patient referral pathways** and **compassionate access programs**.

🟩 Case 2: Intrahepatic Cholangiocarcinoma (iCCA) with IDH1 Mutation

Presented by: Dr. Niyati K. Shah, Medical Oncologist & Hematologist, Sir H. N. Reliance Foundation Hospital

Patient Profile:

- **Age/Gender:** 63-year-old male
- **Diagnosis:** Unresectable intrahepatic cholangiocarcinoma
- **Symptoms:** Abdominal discomfort, elevated LFTs, weight loss
- **Imaging:** Large mass in left liver lobe, segmental ductal dilatation, no distant mets
- **Biopsy:** Adenocarcinoma with biliary phenotype
- **Molecular Testing:** IDH1 R132C mutation detected via NGS
- **Performance Status:** ECOG 1

Treatment Journey:

- **First-Line:** Gemcitabine + Cisplatin + Durvalumab (TOPAZ-based)
- Good disease control for ~7 months
- **Progression:** Mild symptom worsening, growth in existing mass
- **Next Step:** Ivosidenib initiated as targeted second-line therapy

Discussion Points:

- **Importance of Early Molecular Profiling**
  - Dr. Rachna Shroff stressed:
    - IDH1 mutations are **only actionable if caught early**
    - Many patients **never reach second-line** due to clinical deterioration
    - Hence, molecular profiling should be done **at diagnosis**, not after progression
    - Liquid biopsy is a viable option if tissue is inadequate
- **Cost & Access Challenges**
  - Ivosidenib is often **unaffordable without assistance**, with limited availability in India.
  - No local manufacturing or generic alternatives yet.
  - Panel urged pharma partnerships for **named patient programs** and **access bridging**.
- **Managing Complications:**
  - The patient required:

- **Biliary stenting** – multiple exchanges
- **Infection prophylaxis** – for cholangitis prevention
- **Nutrition & electrolyte management**

- Dr. Dhaval Thakkar emphasized:
  - Interventional radiology is vital for multidisciplinary BTC care
  - Training needs to expand beyond metros for timely drainage access

🔸 **Treatment Monitoring**

- CA19-9 levels and imaging were used to monitor progression.
- Panel noted that **clinical progression must supersede biomarker-only changes** when making therapy decisions.

🧠 EXPERT PANEL REFLECTIONS: THE BIGGER PICTURE

👨‍⚕️ Dr. Milind Javle

- "Cholangiocarcinoma isn't rare anymore. It's underdiagnosed, undertreated, and still out of the public eye."
- Urged India to:
  - Build **public registries** for BTC
  - Push for **trial participation in global basket protocols** like ARBOR-BTC
  - Create **funding pools for molecular testing**

👩‍⚕️ Dr. Sewanti Limaye

- Called for:
  - **Equity in access to diagnostics** – tier 2/3 cities are lagging behind
  - Development of **guidelines for Indian practice** – including when to test, refer, escalate
  - Creation of a **centralized tumor board platform** for biliary tract cancers

🧬 Dr. Prashant Kumar

- "GBC and iCCA are genetically distinct. Our problem is treating them the same way because we lack testing."
- Urged all Indian oncologists to consider upfront genomic testing, especially in <50 age group, female patients, or atypical disease courses.

🔬 Dr. Soumil Vyas

- Shared surgical insights:
  - Downstaging from unresectable to resectable is rare but possible
  - Key factors: disease biology, interval shrinkage, patient nutrition, and response
  - Involvement of surgical oncologists **from Day 1** was emphasized

---

🧾 CLOSING REMARKS: FROM CONVERSATION TO CHANGE

🧑‍⚕️ Dr. Milind Javle

- "India is where GBC is most common. Yet we contribute the least real-world data. Horizon can change that."
- Advocated for:
  - Linking major Indian centers for BTC case pooling
  - Pushing government panels for inclusion of *BTC-targeted therapies in insurance*
  - Establishing BTC navigators and AI-assist tools for triage, referral, and trial matching

👩‍⚕️ Dr. Sewanti Limaye

- "Let's move beyond silos. Let's build multidisciplinary, inclusive, and scalable BTC care in India."
- Proposed:
  - Establishment of an **India BTC Registry**
  - **BTC Navigation Fellowship** under Horizon and JCF
  - Patient- and family-centered toolkits in regional languages, built with AI